

GQUENCE

Investor & Partner Briefing

A clinically governed platform and product line for oncology nutrition —
live in an Indian cancer centre, ready to scale.

Confidential · August 2026

A large, underserved market

Cancer incidence in India is rising — nutritional care has not kept pace

15.7 lakh

new cancer cases projected
in India for 2025

1 in 9

Indians likely to develop
cancer in their lifetime

~70%

of cancer patients develop
some degree of malnutrition

300+

oncology centres form the
national care network

Why nutrition is the neglected layer

- Oncology spend concentrates on drugs, surgery and radiation
- Nutrition is delegated to under-resourced dietetics teams
- No standard system links assessment to a delivered product
- Outcomes are rarely measured, so the gap stays invisible

Where GQUENCE sits

Between the oncologist and the patient's daily nutrition — a layer that currently has no owner and no system.

- | | |
|---------------------------|-------------------|
| ● Tertiary cancer centres | primary beachhead |
| ● Private oncology chains | fastest to adopt |
| ● Clinical trial sponsors | data as the draw |

Two revenue lines from one deployment

The platform creates the prescription; the prescription creates the order



Platform licence

Recurring subscription per oncology site. Covers the clinical engine, workflow, monitoring and research module.

Predictable · sticky · scales with sites



Product margin

Every approved prescription becomes a manufactured, batch-tracked product. Weekly reviews generate repeat orders automatically.

Recurring by clinical design, not by upsell

The structural advantage

A weekly prescription cycle means each enrolled patient generates a recurring order for the duration of treatment — without a salesperson, because the reorder is a clinical decision made inside the platform. Software adoption and product revenue reinforce each other.

Traction: not a concept, a running system

Live in an oncology centre with real patients and real manufacturing

45

patients enrolled

118

products manufactured
& dispatched

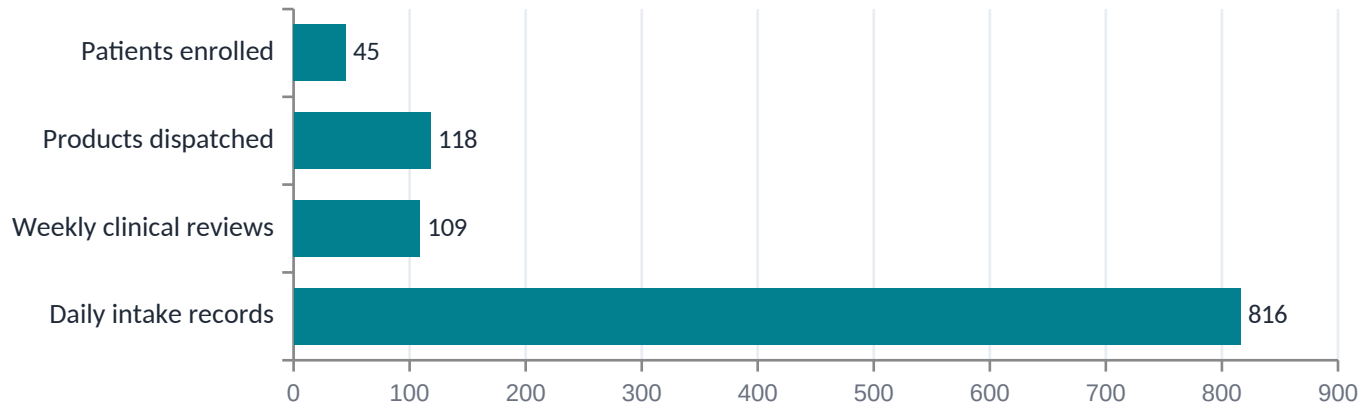
925

clinical records
captured

6

governed user roles
in production

Activity generated by the live deployment



What this de-risks

- Clinicians adopt it in a live clinic
- Manufacturing and dispatch work end to end
- Data accrues without extra staff effort
- The engine handles real case complexity
- Regulatory documentation already drafted

What is hard to copy

The defensibility is in the accumulated clinical logic and the data loop

01

Encoded clinical logic

136 configurable parameters, 16 chemotherapy and radiation interaction rules, 19 safety alerts — built with oncologists over successive iterations, not scraped from a textbook.

02

The data flywheel

Every weekly review adds longitudinal outcome data. That dataset improves the engine and becomes an asset no new entrant can start with.

03

Workflow lock-in

Once prescribing, manufacturing and dispatch run through one governed chain, replacing it means replacing the whole clinical process — not swapping a vendor.

04

Regulatory groundwork

Clinical source documentation and batch traceability already in place, which is where most nutrition products stall.

How the economics work

Structure shown; figures to be confirmed against your final pricing

Per enrolled patient

The economics are driven by treatment duration, because each week of therapy is one manufactured product.

Initial product	1 per patient at enrolment
Weekly products	1 per week of active treatment
Typical pilot course	up to 7 weeks observed to date
Reorder trigger	clinical review, not sales contact

Per site

A single oncology centre carries a standing patient population, so site value compounds as enrolment grows.

Licence	recurring, per site
Product volume	scales with enrolled patients
Marginal cost of a new site	software cost near zero
Expansion path	more patients per site, then more sites

Insert your pricing, COGS and target margin before presenting.

Route to scale

Depth in one centre first, then replication

NOW

Prove the loop

One centre, live. Establish that clinicians adopt it and the supply chain holds.

45 patients

NEXT

Outcome evidence

Larger cohort with structured follow-up to convert engagement data into published results.

Extended pilot

THEN

Multi-centre

Replicate into partner sites. Per-site cohort comparison is already built.

3-5 sites

LATER

Category position

Become the default nutritional layer for Indian oncology, with the dataset as the moat.

Network effect

Each phase de-risks the next: adoption, then evidence, then replication, then category ownership.

Risks, and how they are managed

The ones a diligent investor will raise anyway

Risk	Concern	Mitigation
 Evidence is early	Single-arm pilot with no control group.	Extended cohort with structured follow-up is the primary use of funds.
 Clinical adoption	Doctors resist tools that add work.	Deployed live; the risk panel is designed to save time, not demand it.
 Regulatory	Nutraceutical classification and labelling.	Clinical source documentation and batch traceability already built.
 Manufacturing scale	Batch production must scale with sites.	Store workflow is already multi-site and role-governed in software.

The ask

1

Capital

To fund the evidence phase and manufacturing scale-up:

- Extended clinical cohort and follow-up
- Manufacturing capacity and quality systems
- Clinical and commercial team
- Regulatory and publication work

2

Sites

Partner oncology centres willing to deploy and co-evaluate:

- Clinical champion at each centre
- Shared protocol and outcome definitions
- Per-site cohort comparison already built
- Co-authorship on resulting publications

Insert your funding amount, valuation and target site count before presenting.